Nivolumab · every 2 weeks · previously treated NSCLC

Nivolumab 240 mg IV over 30 minutes OR 600 mg as a subcutaneous injection over 3–5 minutes on Day 1 every 2 weeks. Your team confirms one formulation; the whole visit includes checks and preparation. Continue until progression or unacceptable toxicity, with individual benefit and safety review.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Hormones / nervous system

Nivolumab

Severe headache, confusion, weakness, dizziness or marked thirst need prompt advice.

2 Lungs

Nivolumab

New cough or breathlessness can signal inflammation or another urgent cause.

3 Heart

Nivolumab

Chest pain, racing heartbeat or fainting needs urgent assessment.

4 Liver

Nivolumab

Yellow eyes/skin or dark urine may signal inflammation; tests also check the liver.

5 Kidneys

Nivolumab

Report much less urine, new swelling or a change in kidney tests.

6 Bowel

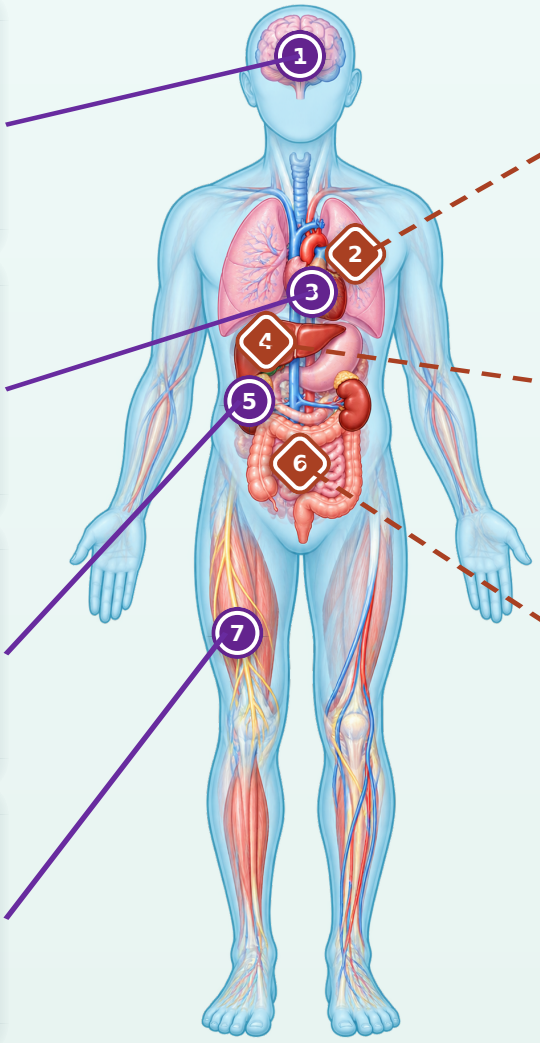
Nivolumab

New or persistent diarrhoea, blood in stools or severe tummy pain needs urgent advice.

7 Skin / muscles

Nivolumab

Spreading rash, blisters, severe weakness or muscle pain needs prompt review.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact the oncology team immediately for new or worsening breathlessness, chest pain, persistent diarrhoea, severe tummy pain, yellow skin/eyes, much less urine, severe headache, marked weakness, confusion, unusual thirst or passing much more urine, or a spreading/blistering rash. For severe breathing difficulty, collapse, seizure or other immediately dangerous symptoms, seek emergency care. Do not wait for the next infusion.

Your treatment, at home.

Important medicine-specific effects

Nivolumab

- Immune inflammation of lungs, bowel, liver, kidneys, hormone glands, skin, heart and nervous system
- Infusion or injection reactions
- Some effects need steroids, hospital care or long-term hormone replacement
- Effects may start after the final dose

Surgery, radiation and monitoring: open the whole-treatment map or scan the QR.

Make the plan practical

Sickness medicines

Nivolumab alone has minimal sickness risk. Routine preventive anti-sickness medicines or premedication are not usually required; the team may prescribe symptom relief.

Immune alert card

Carry your alert card during treatment and afterwards. Show it when seeking urgent care; symptoms can be mistaken for an infection or an ordinary stomach upset.

Hormones and steroids

Some immune effects need steroids or hormone replacement. Follow the prescribed plan and do not stop steroids suddenly. Ask for help if symptoms worsen or you cannot take essential medicines.

Pregnancy and breastfeeding

Discuss pregnancy, fertility and breastfeeding before treatment. Use effective contraception during treatment and for at least 5 months after the last dose. Breastfeeding requires a specialist decision; contact your team promptly if pregnancy occurs.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Complete reference, visual timeline and passport

https://sactcheck.com/patient/00483/

Sources and review status are available online. Follow your own prescription and hospital instructions.